⚡ Clinoma

معسكر الورقة الثانية للأطفال — اليوم الأول

📝 Day 1 Quiz — Second Paper Camp

🖊️ Student Copy
Total Marks: 45 | Time: 30 min

QUIZ TYPE	TOTAL MARKS	DURATION
Interactive Matching	45 Marks	30 Minutes

Matching Questions sets

Total: 45 marks

1 Theme: Acute Upper Airway Infections & Obstruction (5 marks)

A. Viral Croup
B. Acute Epiglottitis
C. Acute Bronchiolitis
D. Foreign Body Aspiration
E. Spasmodic Croup

From the list above, select one choice that is most suitable for the following options:

1 A 6-month-old infant presents during winter with a first episode of expiratory wheezing, tachypnea, and chest retractions, following a 3-day history of coryza and low-grade fever.

2 Anteroposterior neck radiography of a 2-year-old child presenting with a barking cough and inspiratory stridor demonstrates subglottic narrowing known as the "steeple sign".

3 A 3-year-old child presents with a sudden onset of barking cough at night without a preceding viral prodrome or fever, which completely resolves within a few hours.

4 A 4-year-old unimmunized child presents with a toxic appearance, high fever, severe respiratory distress, drooling of saliva, and insistence on sitting forward in a tripod position.

5 A 14-month-old healthy toddler experiences a sudden episode of choking and coughing while playing, followed by a fixed localized wheeze and decreased breath sounds on one side.

No. 1 (1 mark)	No. 2 (1 mark)	No. 3 (1 mark)	No. 4 (1 mark)	No. 5 (1 mark)
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2 Theme: Diagnostic & Management Safety Maneuvers (5 marks)

A. Expiratory chest radiograph
B. Direct oral examination with a tongue depressor
C. Spirometry showing reversibility of airway obstruction
D. Nebulized Epinephrine followed by a 2-hour observation period
E. Routine administration of sedatives

From the list above, select one choice that is most suitable for the following options:

1. A diagnostic evaluation modality used to confirm the diagnosis of Bronchial Asthma by documenting the reversible nature of the airway obstruction.
2. A strictly prohibited procedure in a child suspected of acute epiglottitis due to the critical risk of precipitating fatal laryngospasm before securing the airway.
3. The preferred radiological technique used to visualize obstructive emphysema or localized air trapping caused by a radiolucent foreign body.
4. A fast-acting therapeutic intervention used to decrease subglottic edema in severe croup, requiring monitoring due to the risk of rebound symptom recurrence.
5. A management intervention that is strictly avoided and contraindicated in the standard supportive care of acute bronchiolitis.

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3 Theme: Differential Diagnosis & Pneumonia Mimics (5 marks)

- **A.** Diabetic Ketoacidosis (DKA)
- **B.** Congenital Lung Anomalies (e.g., CCAM/Sequestration)
- **C.** Bronchial Asthma
- **D.** Heart Failure
- **E.** Loffler Syndrome

From the list above, select one choice that is most suitable for the following options:

1. A metabolic condition that mimics respiratory distress by causing deep, rapid respiration (Kussmaul breathing), while the chest examination remains entirely clear.
2. An underlying structural etiology that must be highly suspected in a child presenting with recurrent episodes of pneumonia restricted to the exact same lung site.
3. Characterized by chronic airway inflammation, bronchial hyperresponsiveness, and recurrent acute episodes triggered by exposure to allergens.
4. A non-respiratory systemic condition presenting with tachypnea, cardiomegaly, a gallop rhythm, and a tender enlarged liver, which can be mistaken for childhood pneumonia.
5. Characterized by transient pulmonary eosinophilic infiltrates accompanied by mild respiratory symptoms, often secondary to parasites or drugs.

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4 Theme: Pediatric Pneumonia Pathogens (5 marks)

A. Staphylococcus aureus
B. Mycoplasma pneumoniae
C. Chlamydia trachomatis
D. Viral Pneumonia (e.g., RSV/Adenovirus)
E. Streptococcus pneumoniae

From the list above, select one choice that is most suitable for the following options:

1. A school-aged child presenting with an insidious onset of atypical pneumonia, where treatment with Macrolides is the preferred therapeutic choice.
2. A chest radiograph of a severely ill infant reveals pathogen-specific lung complications such as pneumatoceles, empyema, or pyopneumothorax.
3. Characterized by a laboratory profile showing a normal to mildly elevated white blood cell count with a distinct lymphocyte predominance.
4. The most common cause of bacterial pneumonia, characterized by a high white blood cell count with polymorphonuclear (PMN) predominance and markedly elevated ESR and CRP.
5. An infant presenting with subacute pneumonia, a characteristic staccato cough, and no fever, typically acquired during delivery through an infected birth canal.

No. 1 (1 mark)	No. 2 (1 mark)	No. 3 (1 mark)	No. 4 (1 mark)	No. 5 (1 mark)
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5 Theme: Pneumonia Classifications & Hospitalization (5 marks)

A. Community-Acquired Pneumonia (CAP)
B. Hospital-Acquired Pneumonia (HAP)
C. Ventilator-Associated Pneumonia (VAP)
D. Age < 6 months
E. Fast breathing (Tachypnea)

From the list above, select one choice that is most suitable for the following options:

1. A type of lower respiratory infection that presents clinically in a child more than 48 hours after hospital admission.
2. A specific category of nosocomial pneumonia that characteristically develops more than 48 hours following endotracheal intubation.
3. An absolute age-based indication for hospitalizing a child diagnosed with pneumonia, regardless of the initial mild clinical scoring.
4. Defined as signs and symptoms of pneumonia developing in a previously healthy child due to an infection acquired outside a hospital facility.
5. The primary clinical sign used by the WHO classification to identify pneumonia in a child presenting with a cough or difficult breathing.

No. 1 (1 mark)	No. 2 (1 mark)	No. 3 (1 mark)	No. 4 (1 mark)	No. 5 (1 mark)
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6 Theme: Bronchial Asthma Triggers & Clinical Indicators (5 marks)

A. Insidious onset of acute asthma episodes
B. Sudden onset of acute asthma episodes
C. Pulsus paradoxus
D. Palpable liver and spleen margins
E. Day-to-day and/or morning-to-evening variation of PEF or FEV1 >= 20%

From the list above, select one choice that is most suitable for the following options:

1 The typical clinical onset pattern observed when an acute asthma episode is triggered following viral infections.

2 The typical clinical onset pattern observed when an acute asthma episode is caused by exposure to irritants like cold air or noxious fumes.

3 A cardiovascular clinical sign characterized by an exaggerated drop in systolic blood pressure during inspiration, accompanying severe asthma attacks.

4 A physical examination finding in severe asthma that occurs entirely as a result of chest hyperinflation displacing the diaphragm downward.

5 A diagnostic parameter obtained on lung function tests that indicates significant diurnal variation and airway reversibility in a child with asthma.

No. 1 (1 mark)	No. 2 (1 mark)	No. 3 (1 mark)	No. 4 (1 mark)	No. 5 (1 mark)
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7 Theme: Bronchial Asthma Lab Profiles & Comorbidities (5 marks)

- **A.** Early-stage acute asthma exacerbation
- **B.** Late-stage impending respiratory failure
- **C.** Gastroesophageal Reflux Disease (GERD)
- **D.** Eosinophilia
- **E.** Silent chest

From the list above, select one choice that is most suitable for the following options:

1. A chronic comorbid condition commonly associated with bronchial asthma that induces recurrent micro-aspiration, making the airway disease difficult to control.
2. A blood laboratory finding that serves as a useful indicator of the underlying allergic or atopic profile in a child with bronchial asthma.
3. An arterial blood gas (ABG) profile during an initial asthma attack showing hypoxia, low PCO2, and respiratory alkalosis due to hyperventilation.
4. An arterial blood gas (ABG) profile showing a rising or normal PCO2 accompanied by respiratory acidosis, indicating severe respiratory muscle fatigue.
5. A critical auscultatory finding during a severe asthma attack where air entry is so poor that wheezing disappears, indicating imminent respiratory arrest.

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8 Theme: Acute Bronchiolitis Pathophysiology & Complications (5 marks)

- **A.** Apneic spells
- **B.** Complete plugging of airways by mucus and sloughed cells
- **C.** Humidified oxygen
- **D.** Ribavirin
- **E.** Respiratory Syncytial Virus (RSV)

From the list above, select one choice that is most suitable for the following options:

1. A critical life-threatening complication that must be closely anticipated in young infants below 2 months of age presenting with acute bronchiolitis.
2. The primary supportive therapeutic agent routinely administered in acute bronchiolitis to manage documented hypoxemia.
3. A specific antiviral agent that may be considered for the treatment of acute bronchiolitis in highly selected high-risk or congenital cases.
4. The pathological mechanism in the bronchioles that directly results in hyperinflation, air trapping, and ventilation-perfusion mismatch.
5. The most common underlying viral etiology responsible for the vast majority of cases of acute bronchiolitis in infants.

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9 Theme: WHO Tachypnea Definitions & Critical Airway Rules (5 marks)

A. Respiratory rate >= 50 breaths/minute
B. Respiratory rate >= 40 breaths/minute
C. Avoid placing the child in a supine position
D. Avoid phlebotomy or IV line placement before airway control
E. Respiratory rate >= 60 breaths/minute

From the list above, select one choice that is most suitable for the following options:

1. The strict WHO threshold value used to define fast breathing (tachypnea) in an infant aged 2 months up to 12 months.
2. The strict WHO threshold value used to define fast breathing (tachypnea) in a child aged 12 months up to 5 years.
3. An immediate positioning safety protocol in epiglottitis required because gravity can cause the swollen epiglottis to occlude the airway completely.
4. A critical medical safety restriction in epiglottitis aimed at preventing any painful or stressful interventions that could trigger fatal laryngospasm.
5. The strict WHO threshold value used to define fast breathing (tachypnea) in a young infant less than 2 months of age.

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